

U.S. Health Insurance Coverage Report

Understanding coverage patterns, uninsured rates, and the impact of Medicaid cuts

Olivia Seiler, Masha Antonova, Jack Fang



Project Overview

Understand Coverage

Map how health insurance is distributed across the U.S. : employer, Medicaid, Medicare, non-group, military, and uninsured.

Find Main Drivers

Use correlation analysis to identify which types of coverage are associated with lower uninsured rates across states.

Assess Medicaid Cuts

Model the impact of Medicaid rollbacks on enrollment, and identify which states bear the heaviest burden.



What Each Coverage Type Means

Employer - provided through your job, premiums split between employer and employee

Non-Group - purchased individually on the marketplace (ACA exchanges)

Medicaid - federal/state program for low-income individuals and families

Medicare - federal program for adults 65+ and people with certain disabilities

Military - TRICARE coverage for active duty, veterans, and their families

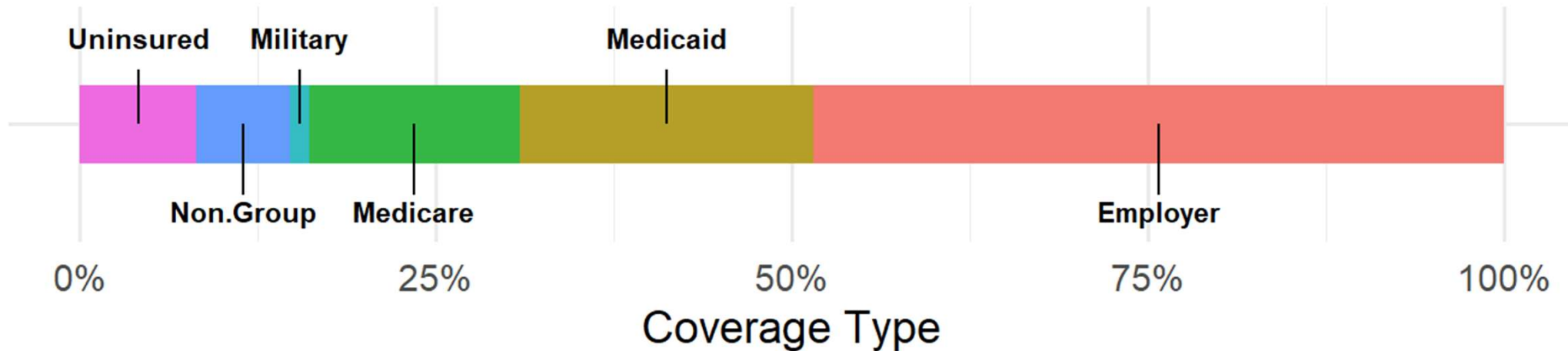
Uninsured - no coverage at all, fully out-of-pocket



How Is the U.S. Covered?

Employer-based insurance dominates but tens of millions remain uninsured

U.S. Health Insurance Coverage Totals

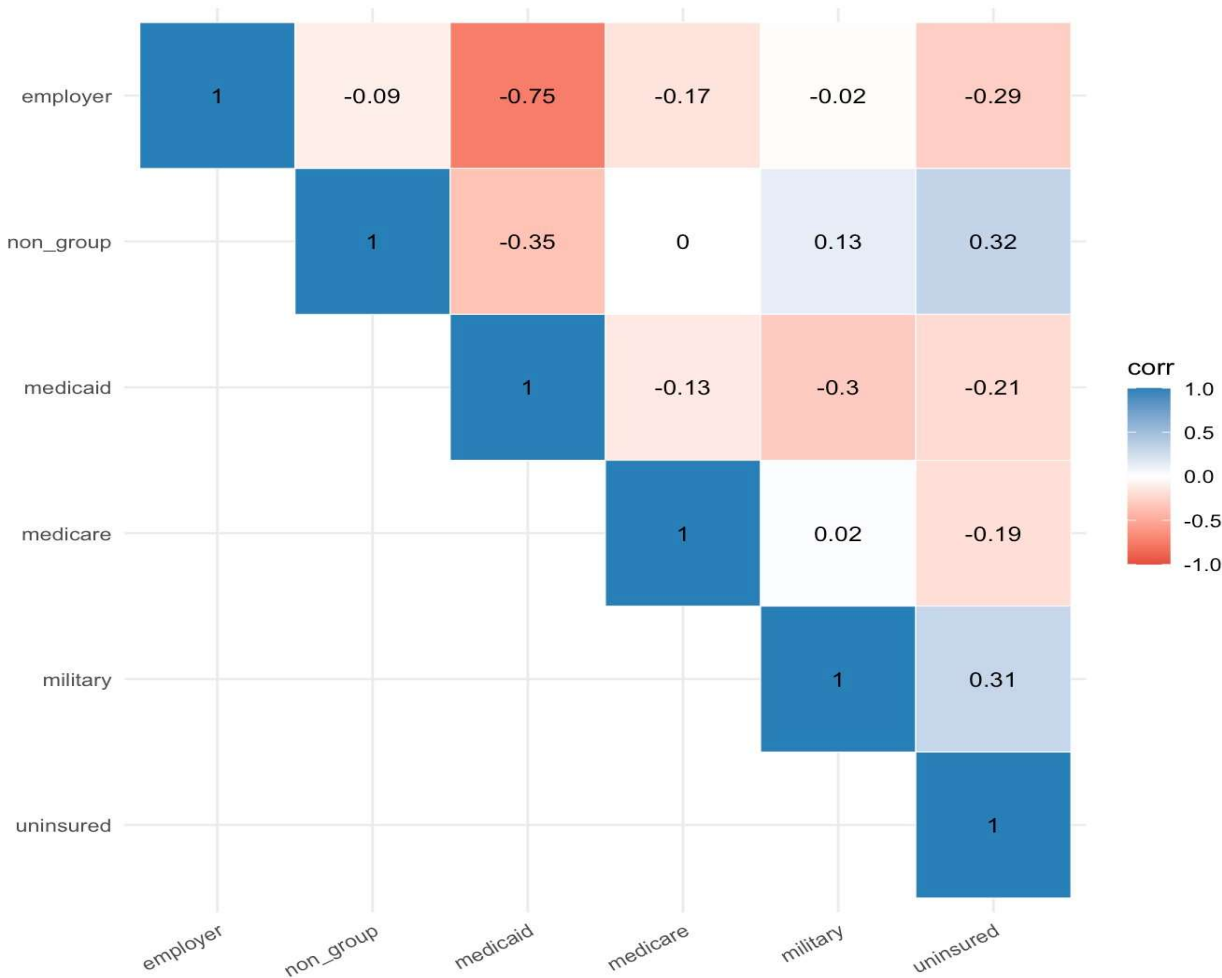


Even though uninsured is not the largest category, it represents a meaningful gap in access to care.



How Coverage Types Relate to Each Other

Lower Triangle Correlation Matrix



-0.75

Employer ↔ Medicaid

An increase in Employer coverage is correlated with a decrease in Medicaid coverage. States rely more on one or the other, rarely both.

-0.21

Medicaid ↔ Uninsured

An increase in Medicaid coverage is correlated with a decrease in uninsurance. Medicaid fills the gap.

+0.32

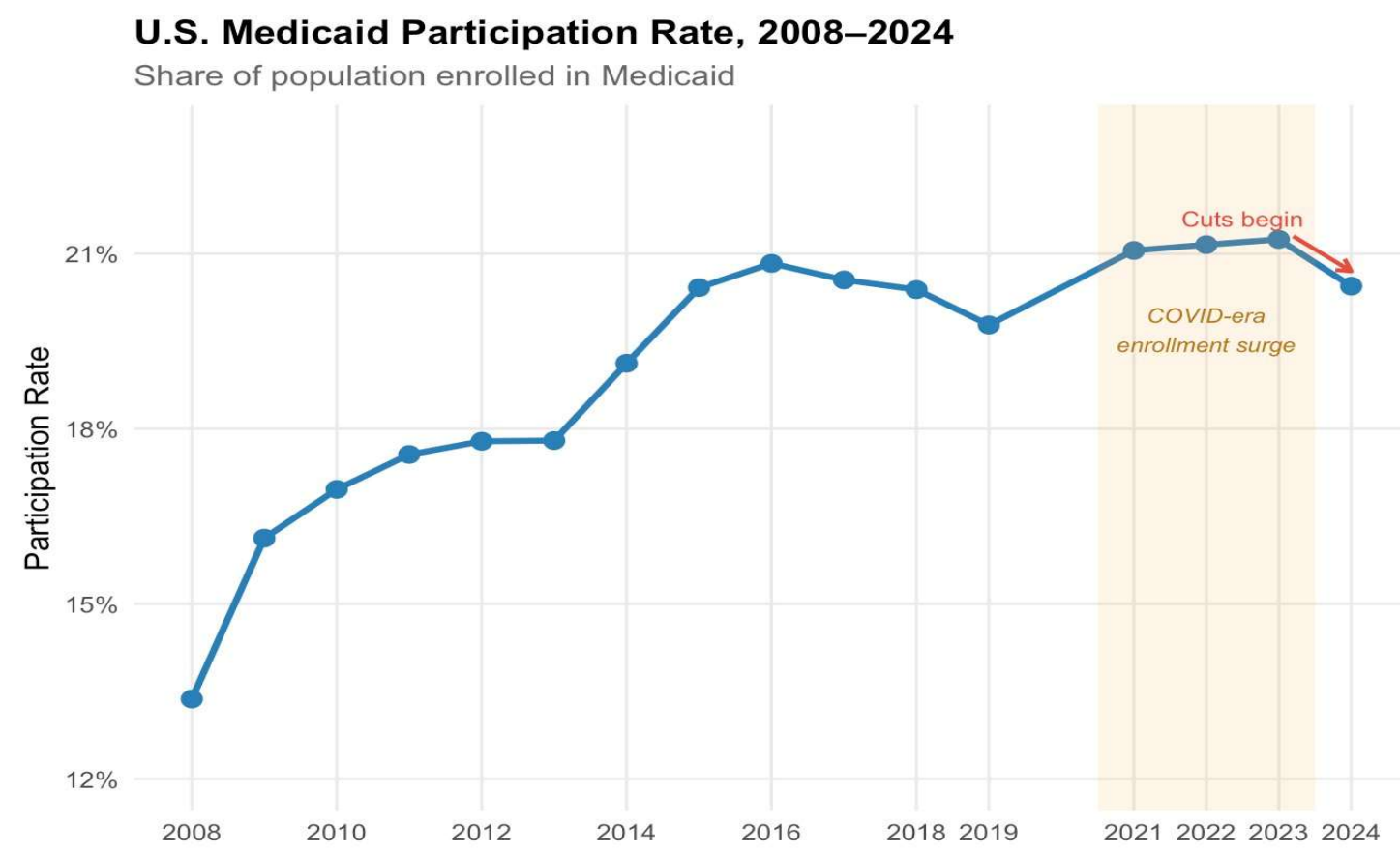
Non-Group ↔ Uninsured

An increase in Non-Group Coverage is correlated with an increase in uninsurance. Non-group alone doesn't fill the coverage gap.



Medicaid Participation: A 16-Year Story

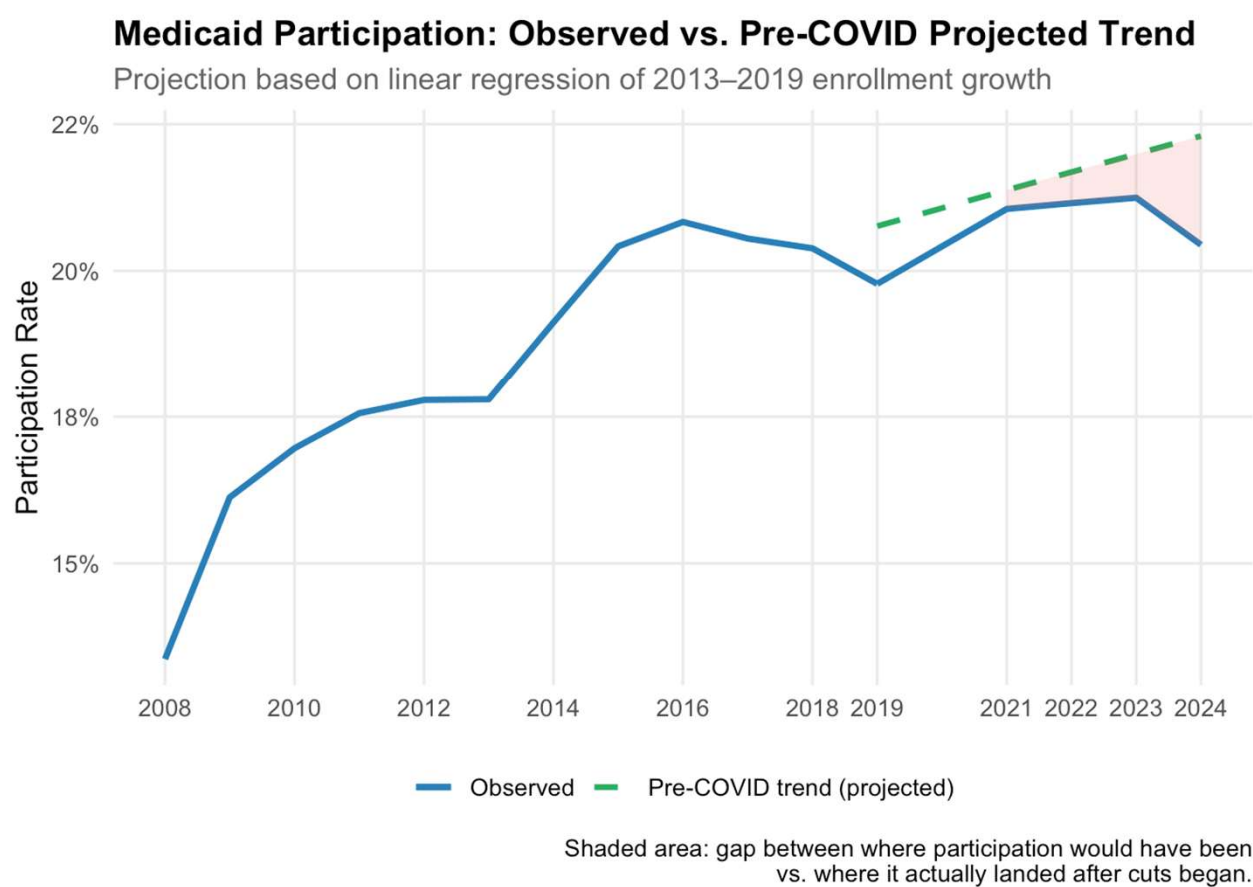
Medicaid Participation Over Time



- Starting in 2023, Congress ended the COVID-era *continuous enrollment* protection
- States were required to resume annual eligibility redeterminations, removing people who no longer qualified or simply didn't respond
- Additional federal funding reductions and state-level policy changes accelerated disenrollment through 2024

Without Cuts, Millions More Would Be Covered

Pre-COVID Projection: What Participation Would Look Like Without Cuts



21.9%

Where participation was projected to reach by 2024 under the pre-COVID trend

20.4%

Where participation actually landed after cuts and redeterminations began

–1.5 ppt gap

Representing roughly 6 million people who fell below the projected trajectory

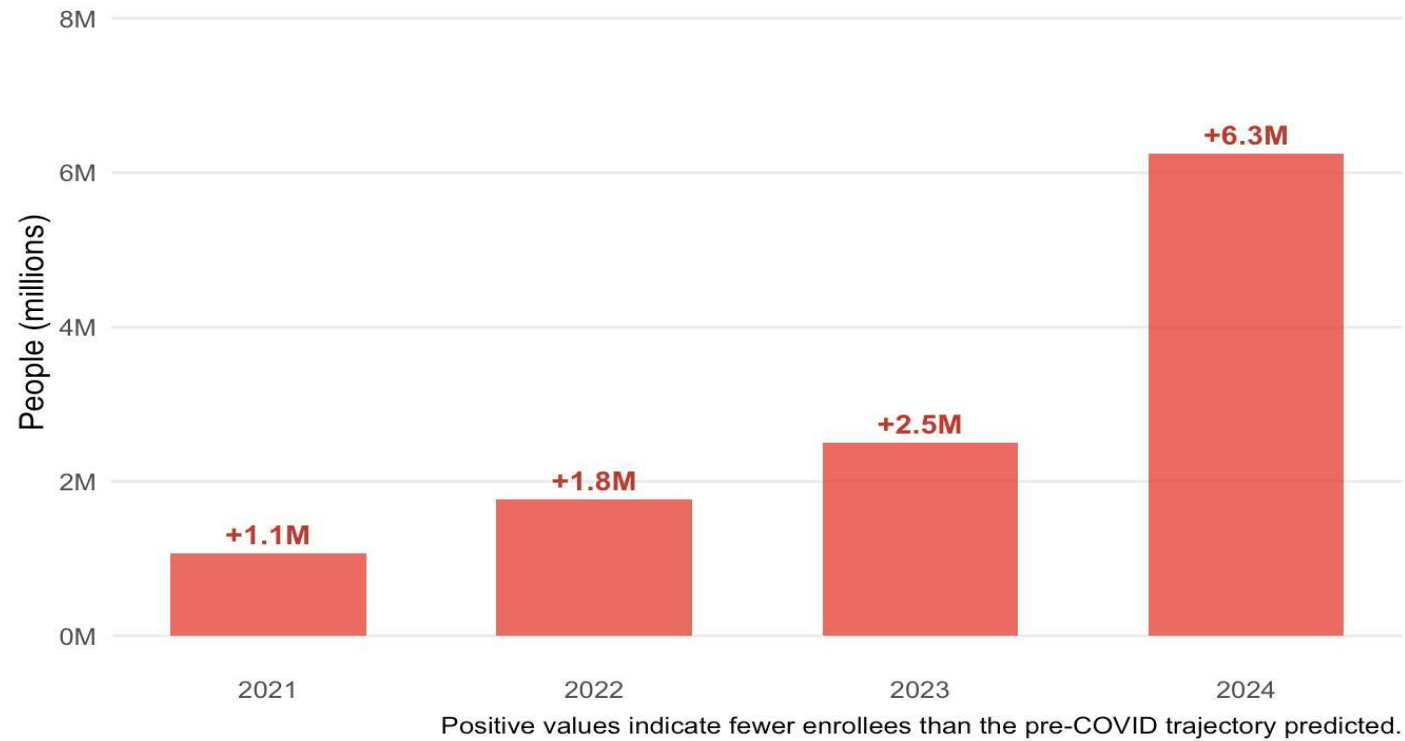


The Growing Coverage Gap

How Many People Lost Coverage?

Estimated People Below Projected Medicaid Enrollment

Difference between pre-COVID trend projection and actual enrollment



6.3M

fewer enrollees by 2024
than projected

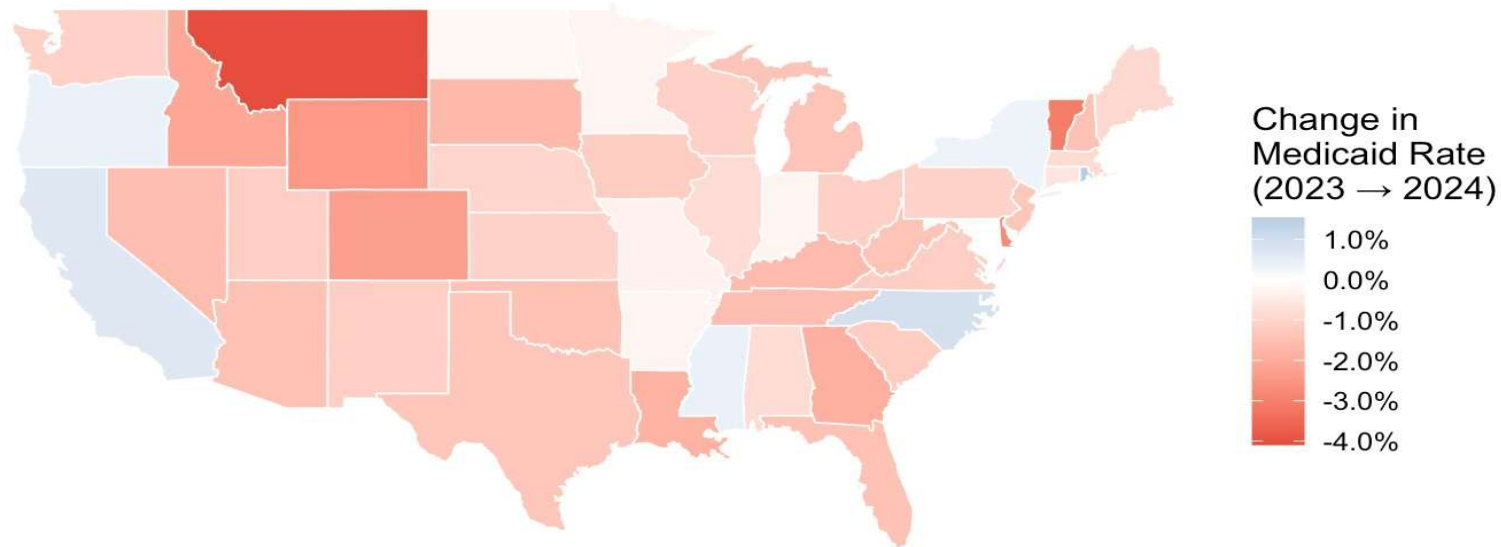


Where Are Cuts Hitting Hardest?

State-level change in Medicaid participation rate, 2023 → 2024 | Red = coverage lost

State-Level Change in Medicaid Participation Rate (2023–2024)

Red = decline (coverage lost); Blue = increase (coverage gained)



Source: KFF State Health Facts

Montana, Vermont, Delaware, Wyoming, and Colorado saw the steepest declines, over 2.5 percentage points in a single year.



Takeaways

01 Employer coverage dominates, but gaps remain

- Employer-based insurance covers the largest share of Americans, yet tens of millions are still uninsured.

02 Medicaid is the strongest buffer against being uninsured

- States with higher Medicaid rates consistently show lower uninsured rates ($r = -0.21$ vs. employer $r = -0.29$ for Medicaid vs employer).

03 Cuts are reversing years of progress

- Since 2024, 6.3 million fewer people are enrolled than the pre-COVID trend projected, and the gap is widening.

04 Geographic impact is uneven

- Montana, Vermont, and several Mountain West states face disproportionate losses, threatening coverage for their most vulnerable residents.



Future Work

Socioeconomic Status

We mostly looked at a general overview on how many people were affected, but was it disproportionate among people of different socioeconomic status?

Who Doesn't Get Employer Coverage?

Most people get Health Insurance coverage through their employer, but who doesn't and why?

Coverage based on Sex

This was another subset of data we had access to and did not explore. Is there disparities in healthcare coverage based on sex?

